

[Portion Redacted]

Interviewer 02:45

All right. So when clients come to you, I know there's bound to be a broad range of issues, but what is usually the presenting issue or the reason that they seek your service?

Therapist 02:57

I think mostly they have issues dealing with differences and also dealing with their own emotions, how to communicate emotions. So the guessing problem you know, you should have guessed what I'm feeling and and difficulty even identifying and communicating emotions think.

Interviewer 03:34

Are there usual Thoris what are sort of differences that the you know, what are the usual differences that they encounter, that you encounter with them.

Therapist 03:48

Cultural differences also the way they they handle like self care as well, like when they're they need some time off. I think usually, woman they want to talk and then then they are more into themselves and quiet and what else I think yeah, like, also needs for, for having fun, as well. So they have to find a balance. One person likes to be out more often and the other is more introverted. And yeah. I can keep searching. But...

Interviewer 05:07

I know it's okay. That's great. Thank you so much. Do you do a formal assessment, with the couples who come in?

Therapist 05:15

Ah, usually I meet them together in the first session. And I asked how they met each other, and what they liked about each other in the beginning, why they got together. And then I talk about the issues that are arising. And I listen to both of them. I asked for the other person's opinion on the subject, if the other person wants to add something,

Interviewer 05:59

Do you use a pen and paper assessments?

Therapist 06:04

if I use pen and paper?

Interviewer 06:06

Pen and paper for scales, for example,

Therapist 06:09

oh, no, not in the first session.

Interviewer 06:14

Or when do you use them, if not in the first session?

Therapist 06:19

I think usually after four or five sessions, and I have some kind of bond with them already, because I find it really distancing to have some sort of assessment like this and and I don't think it's accurate. And, you know, if, if I do this so much in the beginning.

Interviewer 06:52

So what are the scales that you're used in this after that four to five sessions?

Therapist 06:58

I use the scales for IBCT the ones that are in the book at personality assessment, emotion assessment, and also FAP the FIATh cue for functional analytic psychotherapy.

Interviewer 07:20

I'm not very familiar. What's the functional analytic therapy assessment?

Therapist 07:27

Is the FIAT cue I can send it to you. is the Functional Idiographic Assessment Template. This is the one.

Interviewer 07:46

So you have the IBCT scale, that you use too? Are there other skills that you use?

Therapist 07:54

Um, so far that said, I've, I hope I learned more about other skills during the course the externship?

Interviewer 08:11

All right. So how informative do you find these scales?

Therapist 08:18

Um, well, the FIAT cue like a lot. Because it really goes deep in the in the issues from each person, or emotional understanding and conflict daily, and needs, how they communicate their needs. And IBCT. The one that I have at least I find that really short, I think. It is good to work in all during this session.

Interviewer 09:00

And do you have any challenges in using these skills?

Therapist 09:06

No, no, really? It's easy to use, I think.

Interviewer 09:11

And do you pay for these scales?

Therapist 09:15

No, no, no.

Interviewer 09:18

All right. By the way, sorry, Can I ask for, do you have a copy of the the other scale as well?

Therapist 09:31

The IBCT? I have it in the book. It's in the book. The reconcilable differences.

Interviewer 09:41

Okay. All right. I'll take a look at that later. Yeah. All right. So you'll find FIAT cue to be more informative than the IBCT because of the emotional understanding and conflict, conflict style needs, etc. So why do you use IBCT for you like, well, what are you getting out of it?

Therapist 10:05

Sorry, can you ask me again?

Interviewer 10:07

Sure. You said that you find FIAT cue to be more informative than IBCT. I wanted to ask why you use IBCT? Like, why? What you're getting out of it?

Therapist 10:23

I think it's, it's easy for the couples. It's short, and is direct to the point. I think it's really easy for them. So as a first instrument, I think it's good to get like an understanding, a quick understanding and get them to communicate.

Interviewer 10:54

All right, But you also said earlier that you didn't want it to be, you didn't want to give this as a start? Because you felt that it was. It was distancing. And you don't really think it's accurate? Is that correct?

Therapist 11:10

If I use it in the beginning? Because I think in the beginning, they're really like, with a lot of defenses. So if I use it after a few sessions, I think they are more relaxed be themselves. It's what I mean.

Interviewer 11:36

interesting, I wanted to ask as well, and I think I also touched on this in the email. But across the years, different therapists and different researchers have characterized the construct relationship quality, quite differently from each other. I wanted to get your view about it for you, what is relationship quality?

Therapist 11:59

Okay, well, functional analytic psychotherapy, it's based on relationship quality, you know, if you want to have a healthy relationship. So it qualifies as listening, and getting having conscious of your awareness of your own feelings, and feelings of the other person. That's the first step. So knowing yourself, your values. And having good self knowledge, is also important to have the self knowledge of the other person and having courage to be yourself in your relationship, to show yourself and to ask for your needs. To ask for respect for boundaries, courage to love, as well, and be loved. And how you love of course. So those are the main qualities, I would say. And, of course, that characterize like, operationalize many other behaviors and characteristics. But if he, if you want to be more superficial, I think that will be the main point.

Interviewer 13:41

For you, what differentiates a couple with higher relationship quality, and a couple that has low relationship quality.

Therapist 13:55

Yeah, it's interesting, because I think maybe the the main quality, I would say, is to be able to have discussions to talk about what is going right and wrong, because if I think about like two couples that I see, one of them they have, when I started, they had many, many fights, really big fights, but at least they're trying to communicate and they didn't, they didn't let the love evade, like go away, sort of and with, with the therapist, they learned the therapy, they learn how to communicate this and it's going way better and the love still there, attraction, etc. And an another couple that I see even though they're really respectful with each other, because they didn't talk about the issues. They cannot connect anymore, they really having troubles with this connection. So, I would say that the like, the quality to keep the the relationship going is to communicate to have discussions, you know, and safety. But one without the other is not enough to have safety. But now, now discussions about what is going right or wrong?

Interviewer 15:39

Safety and communication are you can't have one without the other. So, would you say that this are the most important things in high quality relationship? Safety and communication?

Therapist 15:52

Yeah, safety, communication. When you can count on the other person.

Interviewer 16:04

All right. And you also said that, if you want to have a high, if you want to have a healthy relationship, you were saying earlier about listening about understanding myself about communicating, etc. So for you, you know, what is for you? Like, what does a healthy relationship look like? How do you know that a couple has a healthy relationship? How do you like when you observe a couple? How do you say, Oh, that's a healthy one, that's a healthy relationship.

Therapist 16:50

I think communicating with respect and willing to work on the relationship willing to work.

Interviewer 17:08

And on the other hand, like, what is a relationship that is not healthy? Like, what does that look like?

Therapist 17:18

Yeah, I think the bit condescendant . Like, yeah, I think that that is really hurtful and painful, I think it's unhealthy for the relationship. When you have these condescendant, tone of voice you know, like, "yeah, that's what she thinks". Sort of disrespectful like that.

Interviewer 17:51

All right. So when I was asking you, for you, like, what is the relationship quality, for you , it is really this set of behaviors, it's listening, understanding yourself, communicating your emotions, having good self knowledge, having good knowledge of the partner, etc. Would that be for you like that? Is there a relationship quality?

Therapist 18:09

Yeah, I think so.

Interviewer 18:13

All right. And that's how you conceptualize relationships often differ from how your clients conceptualize relationships, and does this impact therapy?

Therapist 18:26

Like, the values that I bring?

Interviewer 18:32

Moreover, how you think about relationships, how you conceptualize relationships, versus how they think about relationships, of the concept of relationships that often differ?

Therapist 18:44

Sometimes I think they have a romantic view about relationships and that really, really impacts the quality of the relationship. So I think my work there is to bring some evidence, you know, some to have a discussion about what is a real relationship and what is a romantic view. But usually, that's the main point that we differ. Like the values of the relationship when they are too romantic, you know, too idealized and and I think that that can be a problem during the therapy.

Interviewer 19:48

In what way is it a problem?

Therapist 19:55

It's not real, they're idealizing too much the partner and it's not real, and that way they come across the real deal. It's like, Oh, is that a relationship? Yeah, that's real. That's a relationship and it's fine.

Interviewer 20:16

All right, interesting. And we also know that the relationship is a complex system, it involves so much involves loves,sex, identity, relationship, conflict, satisfaction, all of that. During therapy, how do you decide which one to focus on? Or which one in this complex system to target?

Therapist 20:45

Interesting. I think usually when the couple rings they let me just check the word in English. Yeah, they list, they list out, you know, the main problem. And usually, it's like a collaboration or way to tackle the issues, you know, like, Well, what do you think it's most important to work first?

Interviewer 21:31

Interesting. So how do you then determine when to end couples therapy?

Therapist 21:38

When it's time to end?

Interviewer 21:41

Or when it's I don't know, when the therapy is done for the couple?

Therapist 21:46

Um, well, I didn't come here to this point, because I'm working with couples for a little time. But I think just the same as, like, single therapy, you know, like, with the individual therapy, you start to see each see each other less often. The problems are being solved, they can communicate, and they have the tools to do it, and it's time to wrap up and end.

Interviewer 22:28

And how long are the sessions for your couples therapy?

Therapist 22:33

I do it for one hour. Just as a normal session, you know, 50 minutes to one hour?

Interviewer 22:42

How often do they meet once every week, once every two weeks?

Therapist 22:47

I prefer once every week. But some people, they don't have the time or the money to do it. So once every two weeks, but I'm ready to do it every week.

Interviewer 23:04

All right. And as we also I also wrote in the email, we are currently developing a relationship quality scale that hopefully better represents relationships as a complex system that would help therapists and treatment planning and evaluation as well. How interested would you be in this kind of scale?

Therapist 23:27

Yeah, really interested. For sure. Yeah, I think would be really good to have such a skill.

Interviewer 23:35

And personally, what would you be looking for in this kind of like newly developed relationships scale, how can we make sure basically that this scale would be useful in our practice?

Therapist 23:52

I think as any any other scale to have more...to have like a, like a more objective view of the problem to have also more evidence, I think it's always good to do you know, like comparison before and after maybe

Interviewer 24:22

All right. And if the scale would exist, you will be able to integrate it in your assessment style as well. Would this be something that you would give after four to five sessions as well? If you can imagine this existing?

Therapist 24:44

Maybe or can also be something to do like, for this day starts the therapy, like, really scale or can have like many parts as well as scale with, like a pre therapy and then after a few sessions or post scale would be also nice like to have many parts.

[Portion Redacted]

Interviewer 29:18

Okay. All right. So I wanted to ask as well, of course, you already supported the project through this interview, what would you be interested in further collaborating for the development of the scale?

Therapist 29:33

Sure.

Interviewer 29:34

Perfect. All right. So I'll be emailing you, because what we're planning to do as well as later on, to interview couples themselves about your relationship, relationship quality, etc. And then later on, once we develop the scale, we would be we would want of course, your feedback about what do you think of it? cetera, et cetera. So before we're going to be wrapping up now but before anything, I just like to summarize what we've been talking about the past 30 minutes or so, [portion redacted]. And I asked how the usual presenting issue is that it was usually dealing with differences and dealing with emotions and how they communicate the guessing problem for example, as well as cultural differences, the differences between men and women and like the need for having fun etc. I also asked you about your assessment procedure you said that you meet at a free session you ask how they got together are they like about each other, etc, their relationship history basically and as well as the issues, but And you only give the pen and paper assessment are the scales after four to five sessions because you think now they're more they're gonna get there, it will be more accurate. And you start off alienating or distancing when presented first. So the scale that you use is the IBCT, which contains our personality

assessments, emotion assessment and so on. And for you use this because it's easy for the couple, it's short, and it's a it helps for them to have a quick understanding and get them to communicate. And you also use FIAT Cue, which you like better for you with some more informative ideas with emotional understanding conflicts needs etc. And then we also talked about relationship quality and for you are really basing this as well on the function analytic tradition. So, it's about having healthy relationship is about listening, understanding themselves, understanding the partner their emotions, their values, how they communicate, second involves having good self knowledge having self knowledge and the other person is also the courage to be yourself to ask for your needs to ask for boundaries and the courage to love and be loved. You said that this can also be operationalized and many other behaviors characteristics, if you were to operationalize this I can in what sense would you do so?

Therapist 32:39

To operationalize the behavior? The listening part I think will be like giving feedbacks and like repeating you know, like, what you're saying loci you know, while the other person talk the awareness part to being aware of yourself is to be able to tackle and describe your own emotions and to and the other person is emotion, you know, to be able to describe in an accurate way, like, the events that occurred or and when it occurred, like the antecedents, like, in a very emotional way to love and be loved. I would say to bring safety to, to the relationship as well to it's difficult to think like, purely behavioral for but I think to to, yeah, to not respond with hostility like aggressiveness, you know, to show care, openness, acceptance. To be worthy of trust. Not, judgmental, to say no with respect as well, to be seen, you know, to see the other version to be seen to feel that it's understanding and empathic. Not defensive, not being defensive. All right, um, and the courage, I would say, to be courageous, to show vulnerability to have the courage to express emotions, to ask for your needs to ask for closeness, changes. I think that's it.

Interviewer 35:57

All right. Thank you so much. And then I also asked you about what differentiates composite high relationship quality and low relationship quality. And for you, it's really the main quality is both safety and communication, it should be both not just one or the other, that they have discussions about what to do and to draw. Because even though, for example, they can be respectful, but they don't talk about it, then that's, that's, you know, also not the way to go. So that would also be the obligor away with that. So we were also talking about, you know, like, how can you see that a person is in a like healthy relationship. And then for you, it's really communicating with respect, you know, willing to work it out, no concession. And then I also ask you about how whether the way that you can centralize relationships often differ from how your clients can separate relationships. And for you, it's really the too idealization to romantic view that, you know, this relationship is gonna be this relationship. It's not, it's not, you know, like, an abstract ideal. And I also asked you like, how do you target which in a complex system to target, and for you it's really taking cue from the couple listing their main problems and collaborating our way to tackle these problems. I also asked you how you determine when to end a couples therapy and for you, you're not there yet, because you've only recently started your practice. But you do need them, one are identified once every week, but usually once every two weeks. And you are interested in the scale that we're developing. And because for you, like your wish list for this would be like to have a more objective view of the problem. And like an evidence, it was a comparison before and after therapy. It was there be anything you want to add to that? To our discussion?

Therapist 37:57

No, I think that's all

Interviewer 37:59

Yeah. All right. Now, before we end, I would like to talk to you about how we will handle the data. Of course, as a research project, we want to be as transparent and as verifiable when we eventually ramped it up so that the people can, can trust results. So I wanted to know which of the data sharing option you are more comfortable with? One option is that transcript portions will be shared, none of which can be identified to you. Another option is that the whole transcript we see were a little older, is that the whole transcript can be shared, removed of sensitive and overly identifying information. So it will be anonymized. Although in this option, because of course, with only transcript portions with only portions people might find it hard to gain context. But with a whole transcript there, they might find it easier to understand where people are, where therapists are coming from, although our risk here is that someone who knows you professionally, well, might be able to piece together the details like oh from [Country A], and [6-10] months, and might not, well, we will go back to [6-10] months if they will. But they might narrow it down among their colleagues about who they believe it is, although, again, no sensitive information to be shared. Some other two options, which one would you be more comfortable, more comfortable with?

Therapist 39:27

So either you share the whole transcript, transcript, or you share what is the other option? I didn't?

Interviewer 39:36

Only the portion so if you can imagine it in a spreadsheet, for example, clinical practice, so that will be contained only portions of what you're saying. Relationship Quality, it will only contain portions of

Therapist 39:48

what Yeah, I think this one is best. Yeah.

Interviewer 39:52

All right, transcript portions. All right. I wanted to ask you something but I probably answered for now